How a Week of Help Holds Together

A short guide to coverage, backup and the concerns worth raising.

The help around your mother grew one favor at a time. Your sister started driving her on Tuesdays, the neighbor got into the habit of looking in most mornings, you come on Sundays. It works, and nobody ever sat down and planned it.

This guide explains what an arrangement like that rests on, and what a family can look at while there's no hurry. It explains. It doesn't say whether the arrangement is enough or whether anything is safe.

Most help at home is informal, and it changes

An arrangement like your mother's is the usual kind. The federal Administration for Community Living says about 80 percent of care at home is provided by unpaid caregivers, and that most people can live at home for many years with help from family and friends and from other paid community support. [1]

It is also made out of other people's weeks, and those change: a new job, a move, a helper's own health. AARP's planning guide takes that as given. It says a plan should be open to modification as team members' ability to help changes, and that with a deep and wide team the support is still there when it does. [2]

- For each helper, write one line that begins “This works as long as…”: your sister's Tuesdays stay free, the neighbor stays next door.
- Then ask, and don't assume. “How are Tuesdays going for you?” is an easy question on a quiet week.

What a family can look at

Two things: the support and the home. For the support, write down the help your mother has now, and the parts of the day and night when she's on her own. No guide can tell you how much time alone suits her. That belongs to the people who know her health.

For the home, the CDC's home checklist for older adults is a good list of where to look: the stairs, with their handrails and lights; the floors, and what has to be walked around; the things kept on high shelves in the kitchen; the way from the bed to the bathroom at night; the tub or shower. [3]

A family can write down what it sees in those places. It can't certify a home safe, and no printed list can either.

- Write what you saw, where and when: “Loose handrail, back steps, noticed Sunday.”
- Leave out what it might mean, and what should be done about it.

Coverage, backup and capacity

Three working words do most of the job here. They're ours, not a clinic's. Coverage is who is with your mother, or looking in on her, across the week. Backup is who could step in for a helper who is away. Capacity is what each helper can realistically keep doing alongside their own job, health and family.

A backup doesn't have to be a relative. AARP tells families not to overlook friends, colleagues, clubs and religious or other groups, and says team members need not live nearby or have huge blocks of time to be of value. [2]

On capacity, the Family Caregiver Alliance suggests a family meeting ask what support role each person wants to play and what support the primary caregiver needs. It also notes that when jobs are broken down into very simple tasks, it is easier for people to help. [4] [5]

- Beside each role, write who could cover for a week, or “nobody identified yet.”
- Nobody is a backup until they've said yes. Ask before you pencil anyone in.

The list for the professionals

Start with what has already been said. If her doctor, pharmacist or therapist has given instructions, write down what they say and where the paper is.

Then sort what you've noticed by who can hear it. The CDC's brochure for family caregivers says to talk openly with your mother and her healthcare provider about falls, to keep an updated list of her medications, and to show a provider or pharmacist all of them, including over-the-counter medicines and supplements. [6]

MedlinePlus, from the National Library of Medicine, suggests getting to know her pharmacist, who can help keep track of her medicines. And the Family Caregiver Alliance's advice for talking with a physician is to prepare questions ahead of time and list your most important concerns. [7] [5]

Beside each concern, write who will raise it and with whom. Then date the page. The CDC suggests updating a written care plan every year, or more often after a change in health or medicines, and a family's page can keep the same rhythm. [8]

What this means for your family

- “We all pitch in” becomes a page: who, which day, doing what.
- Each name gets two more lines: what it counts on, and who could cover.
- A blank on the page is something to ask about. It isn't a verdict on the arrangement.
- What you've noticed goes to her doctor, her pharmacist or your care consultant as what you saw. What it means is theirs to say.

What this guide doesn't do

It doesn't say whether your mother's arrangement is enough, whether her home is safe, or who should do what. The figure, the places to look and the advice about helpers and appointments come from the sources below. Coverage, backup and capacity are the Family Care Roadmap's own working words, and the advice about what to write down is ours.

Ready for the next step?

You can now see what your mother's week rests on: who covers it, what each part counts on, and what you'd like a professional to hear. The care interview covers the same ground in a set order, from supervision and falls to medication handling, the home and each helper's capacity, so nothing depends on what happened to come to mind. It turns your answers into a Family Care Profile you can bring to any care conversation.

Create Your Free Family Care Profile ›

WHERE THIS COMES FROM

1. Administration for Community Living, Who Will Provide Your Care?
2. AARP, Prepare to Care: A Planning Guide for Families
3. Centers for Disease Control and Prevention, Check for Safety: A Home Fall Prevention Checklist for Older Adults (STEADI brochure, 2018)
4. Family Caregiver Alliance, Holding a Family Meeting
5. Family Caregiver Alliance, Taking Care of YOU: Self-Care for Family Caregivers
6. Centers for Disease Control and Prevention, Family Caregivers: Protect Your Loved Ones from Falling (STEADI brochure, 2018)
7. MedlinePlus (National Library of Medicine), Caregiving - medication management: MedlinePlus Medical Encyclopedia
8. Centers for Disease Control and Prevention, Steps for Creating and Maintaining a Care Plan